



EVERGREEN COMMUNITY CARE INC.

EMERGENCY PREPAREDNESS

AND BUSINESS CONTINUITY MANUAL

Emergency Response Procedures — Monthly Training Calendar — Drill Schedules — Annual Review

All Residential Care Programs — Children and Youth

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Effective Date	July 1, 2026
Review Date	July 1, 2027 (annual); also upon any serious incident, provincial emergency directive, or licensing order
Applies To	All staff at all Evergreen Community Care residential care programs; Program Coordinators; Program Managers; Executive Director
Legislative Authority	Workers Compensation Act (WCA) and BC OHS Regulation (Part 4 — emergency preparedness); BC Fire Code; BC Building Code; Emergency Program Act (BC); Community Care and Assisted Living Act (CCALA); Residential Care Regulation (RCR) Standard F; MCFD Standards for Staffed Children's Residential Services (SHSS) Standard F.4
MCFD Standards	Standard F.4 — Safety and Emergency Practices; Standard C.1 — Personal Safety; Standard G.3 — Staff Training
OHS Authority	WCA s.115 (employer duty to prepare for emergencies); OHS Reg. s.4.13–4.16 (emergency preparedness); OHS Reg. Part 4 (workplace safety)
Related Documents	RC Program Manual v4 (ECC-RC-POL-2026-003); OHS Program Binder (ECC-OHS-POL-2026-002); Behaviour Support Manual (ECC-BSP-2026-001); PC Manual (ECC-PC-MAN-2026-001); PM Manual



(ECC-PM-MAN-2026-001); Medication Administration Manual (ECC-MED-2026-001)

⚠ This manual governs emergency preparedness at all Evergreen Community Care residential care programs. Every staff member reads and signs the acknowledgement at Appendix J before their first shift. Emergency response procedures are reviewed with every new staff member during facility orientation. Drills are mandatory and documented.



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PART 1 — Foundation: Principles, Governance, and Roles

1. POLICY STATEMENT AND PHILOSOPHY

Evergreen Community Care Inc. is committed to the safety and security of every child and youth in its care and every staff member working in its programs. Emergencies are not hypothetical — they happen. The question is not whether Evergreen Community Care will face an emergency, but whether it will be prepared when one occurs.

This manual establishes the emergency preparedness systems, response procedures, training requirements, and continuous improvement processes for all Evergreen Community Care residential care programs. Preparation means the difference between a crisis that is managed well and one that causes unnecessary additional harm.

Principle	What It Means in Practice
People first — always	In every emergency, the safety of the children and youth in care comes first, then staff safety. Property and operations are secondary considerations at all times.
Prepare before you need to	Emergency procedures are practised and internalized before an emergency occurs. A procedure read in a binder during a crisis is not the same as a procedure that has been drilled.
Calm leadership	Staff who are calm in an emergency lead youth to safety. Training builds the calm competence that prevents panic.
Every youth has a plan	Emergency responses are individualized where needed. Each youth's mobility needs, medical needs, and behavioural responses to emergencies are known and planned for.
Clear authority and communication	In an emergency, everyone knows who is in charge and what the chain of escalation is. Confusion costs time. Time costs safety.
Learn from every event	Every drill, every near-miss, and every actual emergency is reviewed to identify what can be improved. Preparedness is not static.
Regulatory compliance	This program meets or exceeds all requirements of the BC Fire Code, BC OHS Regulation, RCR Standard F.4, MCFD SHSS Standard F.4, and CCALA for emergency preparedness and safety.

2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Requirement Relevant to Emergency	Key Obligations
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	Preparedness	
BC OHS Regulation, Part 4 — Emergency Preparedness and Response	Workplaces must have written emergency response procedures; workers must be trained; drills must be conducted	OHS Reg. s.4.13: written emergency procedures required; s.4.14: workers trained before working alone or in hazardous conditions; s.4.16: drills conducted at regular intervals
Workers Compensation Act (WCA)	Employer holds primary safety accountability; workers have the right to safe work environment and to refuse unsafe work	WCA s.115: organizational OHS accountability rests with the employer; WCA s.3.12: worker right to refuse unsafe work without retaliation
BC Fire Code	All occupied residential facilities must have written fire safety plans; smoke detectors in all sleeping areas; CO detectors; annual fire safety inspection; documented fire drills	Quarterly announced fire drills; annual unannounced drill; fire safety plan reviewed annually; fire safety plan available for inspection
BC Building Code	Structural and means-of-egress requirements for residential occupancy	Minimum two exits from every floor; emergency lighting; door widths; emergency exit signage maintained and illuminated
Community Care and Assisted Living Act (CCALA)	Licensing conditions include emergency preparedness requirements; Licensing Officer may inspect emergency preparedness documentation at any time	Licensing conditions are part of the program's licence; non-compliance may result in licence conditions, orders, or suspension
Residential Care Regulation (RCR) — Standard F.4	Programs must maintain safety and emergency practices; youth must have access to emergency numbers; emergency equipment must be maintained	RCR Standard F.4: fire and emergency practices; equipment maintained; practices documented; drills conducted
MCFD SHSS Standards — Standard F.4, C.1, G.3	Emergency preparedness is an MCFD program standard; staff training includes emergency response; personal safety of youth requires emergency planning	Standard F.4: safety and emergency practices documented and practised; Standard G.3: staff trained in emergency procedures; Standard C.1: environmental safety maintained
Emergency Program Act (BC)	Local governments and provincial agencies have emergency management obligations; residential care programs must align with local emergency management plans	Programs register with local emergency management authority; follow any evacuation orders or shelter-



		in-place directives from provincial or local emergency management
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3. ROLES AND RESPONSIBILITIES IN EMERGENCIES

Role	Emergency Responsibilities
Youth Care Worker (YCW) — First Responder	Implement the emergency procedure immediately; prioritize youth safety; follow the direction of the shift lead; call 911 if directed or if the shift lead is unavailable; account for all youth; document observations; never leave youth unattended in an emergency
Shift Lead — Incident Commander (IC) for facility-level emergencies	Assume command immediately upon discovery or notification of any emergency; direct all staff; implement the appropriate emergency procedure; call 911; notify the on-call supervisor or PC; account for all youth and staff; maintain safety until the emergency is resolved or external authority (fire department, paramedics, police) assumes command; initiate documentation
Program Coordinator (PC) — Secondary IC / Support	Receive notification from the shift lead; escalate to PM; support the shift lead from off-site if they are not on shift; coordinate external resources; notify MCFD; ensure all critical youth information is available to emergency responders; conduct or direct the post-emergency debrief
Program Manager (PM) — Organizational Lead	Receive notification; coordinate the organizational response; notify the ED; liaise with MCFD and the Licensing Officer if required; approve any temporary service changes; lead the post-emergency review; submit CI Reports and WorkSafeBC reports as required
Executive Director (ED) — Organizational Authority	Notified of all significant emergencies; approves any suspension of normal operations; liaises with Board, MCFD, and media (ED only); authorizes any emergency expenditure; ensures organizational continuity
Board of Directors	Receives significant emergency notifications via ED; ensures the organization has adequate emergency preparedness resources; reviews the annual emergency preparedness report
External Authorities — 911 (fire, police, paramedics)	Upon arrival of any 911 service, they assume command of their area of responsibility; staff follow external authority direction; shift lead provides all relevant information (number of occupants, youth with mobility needs, fire location, etc.)

4. EMERGENCY CONTACT DIRECTORY



The Emergency Contact Directory is posted at every telephone in the program, at the main entrance, and in the medication storage area. It is reviewed and updated at minimum quarterly and whenever any contact information changes.

Contact	Number / Details
911 — fire, police, ambulance (all life-threatening emergencies)	911
BC Poison Control (24/7)	1-800-567-8911
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
BC Crisis Line (24/7)	1-800-784-2433
On-Call Supervisor — posted weekly	[See posted weekly on-call schedule]
Program Coordinator — name and cell	[Insert PC name and cell number]
Program Manager — name and cell	[Insert PM name and cell number]
Executive Director — name and cell	[Insert ED name and cell number]
Program Pharmacy — name and number	[Insert pharmacy name and number]
On-Call Physician / Nurse Practitioner	[Insert after-hours prescriber contact]
BC Hydro Outage Reporting	1-888-769-3766
FortisBC Gas Emergency	1-800-663-9911
Local Hospital Emergency Department	[Insert nearest hospital name and address]
Local Police Non-Emergency	[Insert local police non-emergency number]
Building Owner / Property Manager	[Insert name and number]
HVAC / Building Maintenance Emergency	[Insert number]



BC Emergency Management Division	1-800-663-3456
WorkSafeBC — serious incident reporting	1-888-621-7233
Representative for Children and Youth (RCY)	1-800-476-3933



PART 2 — Facility Preparedness

5. FACILITY SAFETY INSPECTION AND MAINTENANCE

The Program Coordinator is responsible for conducting a monthly facility safety inspection using Appendix G. The Program Manager reviews and signs off on the inspection within 5 business days. All deficiencies are assigned a priority level and a target resolution date.

Priority Level	Definition	Resolution Timeline
Priority 1 — Immediate	Imminent risk of death or serious injury; program cannot safely operate with this deficiency	Resolved before the next youth occupies the affected area; if unresolvable, contact PM immediately for service suspension decision
Priority 2 — Urgent	Significant safety risk that does not require immediate closure but must be addressed rapidly	Resolved within 48 hours; PM notified same day of identification
Priority 3 — Routine	Safety concern that should be addressed promptly but does not present an immediate risk	Resolved within 14 days; documented on the inspection report

5.1 Monthly Emergency Preparedness Inspection Items

Inspection Item	Standard
Fire safety — smoke detectors	Tested by pressing the test button; battery replaced annually; all sleeping areas and common areas covered; document test date and result
Fire safety — CO detectors	Tested monthly; one on every occupied floor; within 5 metres of every sleeping area; document test date and result
Fire safety — fire extinguishers	Visual inspection monthly: pressure gauge in green zone; no visible damage; pin and tamper seal intact; annual professional inspection documented
Emergency exits	All designated exits unobstructed and operable; emergency exit signage illuminated or visible; exterior exit path clear
Emergency lighting	Tested monthly; emergency lighting activates on power failure; all exit routes illuminated; battery backup functional
Emergency equipment	First aid kit fully stocked per checklist (Appendix C); AED operational



	and pads in-date; Naloxone kit in-date (where applicable); EpiPen in-date (where applicable)
Emergency contact directory	Posted at all telephones and main entrance; all numbers current; last update date recorded
Evacuation plan	Posted on every floor; assembly point marked; staff can explain the evacuation route for each floor; youth-specific evacuation needs documented
72-hour emergency supply inventory	Reviewed monthly; expiring items replaced; quantities sufficient for all current residents plus staff (Appendix D)
Hot water temperature	Maximum 49°C at all taps (RCR April 2026 amendment); tested with thermometer; documented
WHMIS/SDS binder	All chemicals in use have current SDS; binder accessible to all staff; new chemicals added within 30 days
Emergency documentation package	Youth Emergency Information Cards (Appendix H) current for all residents; package accessible to shift lead

6. EMERGENCY EQUIPMENT AND SUPPLIES

6.1 Required Emergency Equipment — All Programs

Equipment	Requirement	Inspection Frequency
Smoke detectors	In every sleeping room and common area; hardwired preferred; battery backup; interconnected where possible	Monthly test; annual battery replacement (battery models)
Carbon monoxide detectors	On every occupied floor; within 5 metres of sleeping areas; combination smoke/CO acceptable	Monthly test
Fire extinguisher(s)	Type ABC; one per floor minimum; mounted visibly; accessible within 10 metres of any area	Monthly visual; annual professional inspection
Emergency exit lighting	Battery-powered backup; activates on power failure; illuminates all exit routes	Monthly activation test
First aid kit	WorkSafeBC-compliant Level 1 or Level 2 kit as required by facility size; fully stocked per kit checklist	Monthly stock check; restock immediately after any use
AED (Automated External Defibrillator)	Mounted prominently; pads and battery in-date; cabinet alarm functional (where equipped)	Monthly check: pads and battery expiry; annual professional check



Emergency flashlights	Minimum one per floor; LED; batteries checked monthly; stored in accessible known location	Monthly battery check
Emergency phone list	Laminated; posted at every telephone and main entrance; current at all times	Reviewed quarterly; updated within 24 hours of any change
Evacuation plan and floor plan	Posted on every floor; clearly shows exits, assembly point, and fire extinguisher locations	Reviewed annually; updated on any floor plan change
Battery-powered or hand-crank radio	Emergency broadcasts receivable when power is out; BC Emergency Management broadcasts	Monthly test
Naloxone kit	Where any youth is prescribed opioids; accessible to all trained staff; not locked in medication cabinet	Monthly — expiry check; restock after any use
EpiPen	Where any youth has a documented anaphylaxis risk; accessible immediately; not locked in medication cabinet	Monthly — expiry check; replace immediately after use

7. EMERGENCY DOCUMENTATION PACKAGE

Every shift lead has immediate access to the Emergency Documentation Package. This package is maintained by the PC and updated whenever any relevant youth information changes. It is never locked away from the shift lead.

Package Component	Content and Purpose
Youth Emergency Information Cards (Appendix H)	One card per youth in current residence: legal name; date of birth; MCFD social worker and contact; emergency contacts; allergies and medical alerts; medications currently prescribed; mobility needs; behavioural responses to emergencies; communication needs (AAC, language, cognitive level)
Emergency Contact Directory (current)	All emergency contacts per Section 4; confirmed current within the last quarter
Current medications list	All current medications for all residents; critical timing; prescriber contacts
Copy of youth ISP summary (key health and safety information)	Health and safety highlights from each youth's ISP; not the full ISP — the information emergency responders need immediately
Facility floor plan with evacuation routes	Current floor plan showing all exits, evacuation routes, assembly point location, and any room-specific notes (e.g., non-ambulatory youth location)



Insurance and building information

Property insurance policy number and after-hours claims contact; building owner contact; utility shutoff locations

8. EVACUATION PLANNING AND ASSEMBLY POINTS

8.1 Evacuation Plan Requirements

- A written evacuation plan is prepared for every Evergreen Community Care program facility
- The evacuation plan includes: floor plan of every floor; primary and secondary evacuation routes from every room; assembly point location (outside, away from the building, accessible); designated role for each staff position during evacuation
- The assembly point is selected to be: at least 15 metres from the building; accessible by wheelchair and mobility aids; visible from the facility; away from fire apparatus access routes
- The evacuation plan is posted on every floor in a visible location; all staff can locate and explain the plan for their area

8.2 Youth-Specific Evacuation Needs

Every youth's individual evacuation needs are documented on their Emergency Information Card (Appendix H) and reviewed at every ISP update. The following categories of need are assessed for every youth:

Need Category	Assessment and Planning
Mobility	Can the youth evacuate independently by stairs and on foot? If not: what assistance is required? Is a wheelchair or mobility aid involved? Which staff member is assigned to assist this youth during evacuation?
Cognitive and communication	Does the youth understand evacuation instructions? What language, communication system, or approach is required to direct this youth safely? Does the youth require a familiar staff member to remain with them?
Behavioural response to emergency	How is this youth likely to respond to a fire alarm, sirens, or the chaos of an emergency? What approaches help them cooperate with evacuation? What do staff need to avoid?
Medical needs during evacuation	Does this youth require any medication or equipment during evacuation (e.g., rescue inhaler, insulin)? What is the procedure for ensuring medications and equipment are taken?
Unsafe during emergency behaviour	Is this youth at risk of elopement during an emergency? Are they at risk of self-harm in response to the alarm or chaos? What specific additional supervision is required?



8.3 Non-Ambulatory Youth — Evacuation Chair or In-Place Protocol

Where any youth cannot be evacuated via stairs without specialized equipment, the program maintains an evacuation chair or documents a specific lift-assist procedure. The PC confirms that sufficient trained staff are scheduled on every shift to evacuate all non-ambulatory youth. Where evacuation is not possible via the standard route, a 'defend in place' protocol is established with the fire department — the specific room, its fire resistance, and the communication procedure with the fire department are documented.



PART 3 — Fire Emergency

9. FIRE PREVENTION

Fire Prevention Requirement	Standard
No smoking policy	Smoking is prohibited inside the facility and within 6 metres of any entrance, window, or air intake; smoking materials disposed of in designated exterior receptacles
Kitchen safety	Cooking areas never left unattended with heat on; grease traps cleaned regularly; kitchen fire suppression equipment maintained; no deep-fryers without approved ventilation
Electrical safety	Extension cords are temporary only; no daisy-chaining of power bars; electrical panels accessible and unobstructed; any sparking, flickering lights, or burning smell reported immediately to PC
Storage and clutter	Exit routes and corridors maintained clear at all times; combustible materials stored away from heat sources; no storage within 1 metre of electrical panels
Candles and open flame	Candles and other open flames require PM approval and may only be used in designated circumstances (e.g., cultural ceremony with supervision); never left unattended
Dryer lint	Dryer lint trap cleaned before every load; dryer duct inspected annually
Seasonal decorations	Artificial trees preferred; no real trees if fire suppression is not equipped; decorations not placed near heat sources; all holiday lights CSA certified

10. FIRE RESPONSE PROCEDURE (RACE / PASS)

Upon discovery of any fire or smoke, staff immediately follow the RACE procedure. Every staff member memorizes RACE before their first unsupervised shift. It is posted in every room.

RACE — Fire Response	Action
R — RESCUE	Remove anyone in immediate danger from the fire area. Take only one attempt. Do not re-enter a burning room.
A — ALARM	Activate the nearest fire alarm pull station. Call 911 immediately — do not wait to confirm the fire. State your address, number of occupants,



	and whether anyone is trapped.
C — CONTAIN	Close all doors between the fire and the rest of the building. Closed doors slow fire spread dramatically. Do not lock doors — they must be openable from the inside.
E — EVACUATE	Evacuate all youth and staff to the designated assembly point using the evacuation plan. Take the Emergency Documentation Package. Account for all persons at the assembly point.

If a fire extinguisher is immediately available and the fire is small (wastebasket-sized or smaller), a staff member who has been trained in extinguisher use may attempt to extinguish the fire using the PASS technique — only after RACE has been initiated (alarm pulled, 911 called) and only if they have a clear exit route behind them.

PASS — Fire Extinguisher Use	Action
P — PULL	Pull the safety pin from the handle
A — AIM	Aim the nozzle at the BASE of the fire — not the flames
S — SQUEEZE	Squeeze the handle to discharge the extinguishing agent
S — SWEEP	Sweep the nozzle from side to side at the base of the fire until it is out or the extinguisher is empty

⚠ NEVER use a fire extinguisher on a fire that has spread beyond a single small object or that is producing significant smoke. NEVER re-enter a burning building. If the fire is not extinguished within 10–15 seconds of PASS attempt, abandon the attempt and evacuate immediately.

11. EVACUATION PROCEDURE

Step	Action	Responsible
1. Sound the alarm	Activate the nearest fire alarm pull station if not already activated; call 911	First staff to discover fire or alarm activation
2. Assign tasks	Shift lead assigns: staff to specific youth; staff to retrieve Emergency Documentation Package; staff to sweep each floor	Shift lead immediately
3. Evacuate youth	Move all youth to the nearest safe exit; close doors as you go; do not use elevators; assist	All staff; each assigned to



	youth with mobility needs per their Emergency Information Card	specific youth
4. Sweep each floor	Designated staff check all rooms, bathrooms, closets, and common areas on their assigned floor; knock on doors and announce 'FIRE — EVACUATE'; close doors after checking	Designated sweep staff — do not delay evacuation of other youth
5. Proceed to assembly point	All youth and staff proceed to the designated assembly point; do not re-enter the building for any reason	All staff and youth
6. Account for all persons	Shift lead accounts for every youth and staff member at the assembly point using the Emergency Documentation Package; identify any missing person immediately to emergency services	Shift lead
7. Meet the fire department	Shift lead meets the incoming fire department at the entrance; provides: address; number of occupants; any missing persons; location of fire (if known); any hazards (oxygen, medications, propane)	Shift lead
8. Do not re-enter	No staff or youth re-enters the building until the fire department declares it safe	All staff
9. Notify PC and PM	Shift lead calls PC or PM immediately after accounting for all persons	Shift lead
10. Document	Document the incident: time of alarm; time of evacuation complete; any injuries; fire department actions; all-clear time	Shift lead; PC follows up with formal documentation

12. FIRE DRILL REQUIREMENTS AND DOCUMENTATION

Drill Requirement	Standard
Quarterly announced fire drills	Minimum one announced fire drill per quarter (4 per year); all staff and youth who are present participate; simulate actual evacuation to assembly point; timed from alarm activation to all persons accounted for at assembly point
Annual unannounced fire drill	Minimum one unannounced fire drill per year; conducted at night or on a weekend to simulate non-peak hours; PM or PC observes and evaluates; all findings documented
New staff fire drill orientation	Every new staff member is walked through the fire evacuation procedure and shown all exits and the assembly point during their first



	shift; they participate in the next scheduled drill
Documentation	Every fire drill is documented on the Fire Drill Record (Appendix E): date; time; announced or unannounced; number of staff and youth who participated; time from alarm to all persons at assembly point; any issues identified; corrective actions taken
Post-drill debrief	Brief debrief with all participating staff after every drill: what went well; what needs improvement; any youth-specific issues; corrective actions assigned with owner and date
Corrective actions	Any deficiency identified in a fire drill is assigned a corrective action with a named owner and target date; reviewed at the next drill or sooner



PART 4 — Natural Disasters

13. EARTHQUAKE RESPONSE

BC is in an active seismic zone. Earthquakes can occur without warning. Staff must know the response before one occurs.

13.1 During the Earthquake — Drop, Cover, Hold On

Instruction	Detail
DROP	Drop to the floor immediately; do not run
COVER	Get under a sturdy desk or table; if no table is available, cover your head and neck with your arms; move to an interior wall away from windows
HOLD ON	Hold on to your shelter until the shaking stops; do not let go; the shaking may last longer than it feels
Stay indoors	Research shows most injuries occur when people try to move during shaking; stay where you are until the shaking stops completely
Away from windows	Windows and glass are the greatest hazard; move away from windows, exterior walls, and anything that could fall
Youth	If possible: get the youth under cover; if the youth is in a bed, stay in bed and protect the head and neck with a pillow

13.2 After the Earthquake

Step	Action	Responsible
1. Account for all youth and staff	When shaking stops: account for all youth and staff; first aid for any injured persons; call 911 if any serious injury	Shift lead
2. Check for hazards	Check for: gas smell (leave immediately if detected); structural damage; fallen objects blocking exits; fire; utility hazards (downed lines outside)	Shift lead + available staff
3. Evacuate if unsafe	If there is any structural damage, gas smell, or fire: evacuate using the evacuation plan; do not re-enter until cleared by fire department or	Shift lead



	structural engineer	
4. Expect aftershocks	Aftershocks can occur for days or weeks after a major earthquake; staff and youth remain prepared to Drop, Cover, Hold On again	All staff — communicate to youth
5. Notify chain of command	Shift lead notifies PC; PC notifies PM; PM notifies ED; all follow the CI reporting chain for any injuries or significant damage	Shift lead initiates
6. Do not use landlines for non-emergency calls	Landlines may be overloaded; keep lines clear for emergency use; use cell phones for internal communication	All staff
7. 72-hour supply readiness	If utilities are disrupted: access the 72-hour emergency supply inventory (Appendix D); ration water and food appropriately	Shift lead

13.3 Earthquake Preparedness — Facility

- All heavy objects (bookshelves, cabinets, TVs) are secured to walls with appropriate brackets or straps
- Water heater is strapped to the wall
- Heavy objects are stored on lower shelves; breakables secured
- Gas shutoff location known to all shift leads; posted on Emergency Contact Directory
- 72-hour emergency supplies maintained and inventoried monthly (Appendix D)

14. FLOOD AND SEVERE WEATHER RESPONSE

14.1 Flood Response

Flood Stage	Action	Decision Authority
Flood WATCH (flooding possible)	Monitor Environment Canada alerts; review evacuation route (may become impassable); confirm 72-hour supplies are stocked; notify PM	PC monitors; PM decides
Flood WARNING (flooding expected)	Prepare to evacuate; move essential records and medications to upper floors; notify PM and ED; await direction from local emergency management	PM decides; ED informed
Evacuation Order (mandatory)	Evacuate immediately to the designated alternate facility (see Section 34); take Emergency Documentation Package; medications; youth clothing and essentials; notify MCFD	Shift lead initiates; PM confirms destination; ED authorizes



Active flooding — water in facility	Evacuate immediately; turn off electricity if safe to do so; do not walk through moving water; call 911; notify PC and PM	Shift lead — immediate evacuation
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14.2 Severe Weather — Windstorm, Snowstorm, Extreme Cold

- Monitor Environment Canada alerts (weather.gc.ca); sign up for Emergency Alerts for your region
- Before severe weather: confirm 72-hour supplies are adequate; confirm vehicles are fuelled; confirm staffing plan if roads become impassable
- During windstorm: youth stay inside; windows and doors closed; away from windows; outdoor objects secured
- During snowstorm or extreme cold: confirm heating is functional; if heating fails, see Section 17 (Power Failure); confirm alternative heating source is available
- Staff unable to travel: activate Emergency Staffing protocol (Section 31); shift lead cannot leave until relief arrives

15. EXTREME HEAT AND AIR QUALITY EVENTS

15.1 Extreme Heat Response

BC's extreme heat events (as experienced in 2021) carry significant health risks, particularly for youth on certain medications (antipsychotics, stimulants) that impair heat regulation.

Heat Response Action	Standard
Heat alert notification	BC HealthLinkBC or local health authority heat alert: PC notifies all staff; PM approves any protocol changes
Medication review	PC reviews all current medications for heat sensitivity (antipsychotics, anticholinergics, diuretics, stimulants); consult pharmacy or prescriber for any monitoring protocol during extreme heat
Cooling measures	Fans and air conditioning maximized; cool water and electrolyte beverages freely available; light clothing; cool showers or baths available; avoid outdoor activities during peak heat (11 AM – 4 PM)
Temperature monitoring	Indoor temperature monitored and documented every 2 hours during a heat event; if interior temperature exceeds 30°C: PM notified; alternative cooling location identified
Signs of heat illness	Staff monitor all youth for: heavy sweating; cold/pale/clammy skin (heat exhaustion) or hot/red/dry skin (heat stroke); rapid or weak pulse; nausea; confusion; loss of consciousness. Heat stroke: call 911 immediately; cool the youth rapidly with wet cloths or immersion; this is



	a medical emergency.
Alternative cooling location	PM identifies an accessible air-conditioned location (community centre, library, mall) as an alternative if the facility cannot be adequately cooled; authorization required from PM before transporting youth

15.2 Poor Air Quality — Wildfire Smoke or Industrial Hazard

- Monitor Air Quality Health Index (AQHI) at airqualityontario.ca or BC government air quality resources
- AQHI 7 or above (High Risk): youth remain indoors; windows and doors closed; recirculate AC rather than fresh air intake; outdoor activities cancelled
- Youth with respiratory conditions (asthma, COPD): ensure rescue inhalers are accessible; monitor more frequently; contact prescriber if symptoms worsen
- Air purifiers with HEPA filters in youth bedrooms during extended poor air quality events

16. WILDFIRE AND EVACUATION ORDERS

Alert Level	Meaning	Required Action
Wildfire WATCH	Fire conditions exist that may require evacuation	Monitor; review evacuation plan; confirm 72-hour supplies; notify PM
Evacuation Alert	Evacuation may be required on short notice; be ready to leave in minutes	Pack the Emergency Documentation Package; prepare youth go-bags; confirm alternate facility destination with PM; notify MCFD social workers
Evacuation Order	Immediate evacuation required — life threat	Evacuate immediately to pre-identified alternate facility; take medications, documentation, and youth essentials; call PM and MCFD simultaneously; emergency services lead

The alternate facility for each program in the event of evacuation is identified in the Business Continuity section (Section 34) and is confirmed annually. The alternate facility contact information is in the Emergency Documentation Package.



PART 5 — Facility Emergencies

17. POWER FAILURE

Duration	Response	Action
Short-term (under 4 hours)	Emergency lighting activates automatically; flashlights deployed; continue normal operations with adaptations; report outage to BC Hydro (1-888-769-3766); monitor status	Shift lead manages; notify PC if outage is unexpected or related to facility damage
Extended (4–12 hours)	Activate battery radio for emergency broadcasts; check refrigerated medications — do not open refrigerator unnecessarily; do not cook on the gas stove during a power outage if you suspect a gas leak; notify PC and PM; consider activating alternate heating if needed	PC notified; PM advises on operations
Prolonged (over 12 hours)	PM decides whether to activate Business Continuity Plan (Section 34); consider transfer to alternate facility; notify MCFD; arrange for generator or alternative power for critical equipment; contact property manager	PM decides; ED informed; MCFD notified

17.1 Medications During Power Failure

- Refrigerated medications: a refrigerator that remains closed maintains safe temperature for 4 hours; a cooler with ice extends this; after 4 hours without power, contact pharmacy before administering any refrigerated medication
- Electronic medical equipment (feeding pumps, nebulizers): confirm battery backup status; contact prescriber if equipment cannot operate
- Electronic security systems: confirm that medication cabinet is still secure; manual monitoring if electronic alarms are offline

18. GAS LEAK AND HAZARDOUS MATERIAL RELEASE

18.1 Gas Leak Response

Step	Action
1. Detect	Signs of gas leak: smell of rotten eggs or sulphur; hissing sound near appliances or pipes; dead vegetation near gas lines outdoors; you feel dizzy or nauseated inside



2. Do NOT switch lights on or off	Any spark can ignite gas; do not operate any electrical switches, light switches, or thermostats
3. Do NOT use phones inside	Move outside before calling; cell phones can spark
4. Evacuate immediately	Get everyone out of the building; leave doors open as you go; do not stop to collect belongings
5. Shut off gas at the meter	Only if the shutoff is outside and accessible without re-entering; do not shut off if you are not trained
6. Call FortisBC Gas Emergency from outside	1-800-663-9911 — from outside the building; also call 911 if anyone is injured or the leak is large
7. Do not re-enter	Do not re-enter until FortisBC and the fire department declare the building safe
8. Notify PC and PM	From outside; PC notifies PM; PM decides on service continuity and alternate accommodation

18.2 Other Hazardous Material Release

- Chemical spill or release inside the facility: evacuate the affected area; identify the substance from the WHMIS SDS binder; call 911 if the substance is toxic, flammable, or released in quantity; do not re-enter until safe
- External hazardous material incident (transportation accident, industrial): follow the Shelter-in-Place or Evacuation direction from emergency services; close windows and doors for shelter-in-place; turn off ventilation that draws outside air

19. STRUCTURAL DAMAGE AND UNSAFE BUILDING

- Following any significant structural event (earthquake, major wind event, vehicle impact, flooding): shift lead assesses visible structural damage before allowing re-occupancy
- Signs requiring immediate evacuation: visible cracks in load-bearing walls; roof sagging or damage; foundation movement; floors or walls leaning or buckling; any structural member that has moved, shifted, or cracked
- Following evacuation for structural reasons: do not re-enter until a structural engineer or the fire department declares the building safe
- Notify PM immediately; PM contacts property owner/manager and arranges structural assessment
- If the building is declared unsafe for occupancy: activate Business Continuity Plan (Section 34) for alternate accommodation



20. INTRUDER, LOCKDOWN, AND SECURITY THREATS

20.1 Unauthorized Person at the Facility

Situation	Response
Unknown person at the door — not a known visitor	Do not open the door; identify the person through the window or intercom; ask for identification and purpose of visit; if the visit is not expected and authorized, do not admit; ask the person to contact the PC or PM
Person demanding entry — angry or threatening	Do not open the door; call 911 immediately if the person is threatening or attempting to force entry; call PC; keep all youth away from doors and windows; document the incident
Unauthorized person inside the facility	Do not confront; call 911; get all youth to a safe room; lock the door if possible; wait for police

20.2 Lockdown Procedure

A lockdown is activated when there is an immediate threat to the safety of youth and staff inside the facility from an external or internal threat.

Lockdown Step	Action
1. Alert all staff	Announce lockdown clearly; specific codeword if established; all staff implement immediately
2. Lock all exterior doors and windows	Every entrance secured; windows in common areas closed and covered if possible
3. Move all youth to interior rooms	Away from exterior windows and doors; interior hallways or rooms with locking doors preferred
4. Account for all persons	Account for every youth and staff member; identify anyone unaccounted for immediately to police when they arrive
5. Call 911	Call 911 if not already done; advise: number of people; nature of threat (if known); whether anyone is injured; the address
6. Do not open doors for anyone	Not for parents, visitors, or anyone without police confirmation — police will identify themselves and direct you
7. Stay low and quiet	Stay away from doors and windows; keep youth calm; mobile phones on silent
8. Await ALL CLEAR from police	All clear is given verbally by police in person or by the PC/PM upon confirmation from police; do not unlock until you receive the all clear



21. WATER DISRUPTION AND CONTAMINATION

Situation	Response
Water service disruption (no running water)	Activate 72-hour water supply (Appendix D — minimum 4 litres per person per day); notify property manager; notify PC; if disruption exceeds 24 hours: PM activated; consider alternate facility if drinking water cannot be safely supplied
Boil Water Advisory	All water used for drinking, brushing teeth, making ice, or cooking must be boiled for at least 1 minute (3 minutes at altitude) or bottled water used; notify all youth and staff; monitor media for advisory end notification; document all precautions taken
Water contamination — chemical or biological	Do not use water for any purpose; notify public health authority (1-800-665-4347 for BC); follow all public health direction; activate alternative water supply; contact PM immediately
Plumbing failure — sewage backup or major leak	Isolate the affected area; contact property manager immediately; if sewage is in contact with youth living or sleeping areas, consider temporary relocation; notify PC



PART 6 — Medical and Clinical Emergencies

22. MEDICAL EMERGENCY RESPONSE

Emergency Type	Immediate Response
Loss of consciousness or unresponsive youth	Call 911 immediately; check responsiveness — tap shoulder and shout; if no response: check for breathing; if no breathing or only gasping: begin CPR if trained; send another staff member to retrieve the AED; do not leave the youth; call PC
Cardiac arrest — not breathing, no pulse	Call 911; begin CPR immediately (30 compressions, 2 breaths; compressions only if untrained in breaths); retrieve and apply AED as soon as it arrives; follow AED prompts; do not stop CPR until AED is ready, paramedics arrive, the youth regains consciousness, or you are physically unable to continue
Choking — conscious	Ask 'are you choking?' — if the youth cannot speak, cry, or cough effectively: 5 back blows between the shoulder blades; 5 abdominal thrusts (Heimlich); alternate until the object is expelled or the youth becomes unconscious. If unconscious: call 911 and begin CPR
Seizure	Call 911 if: first seizure; seizure lasts more than 5 minutes; youth does not regain consciousness between seizures; youth is injured. Protect the youth from injury (clear the area); do not restrain; do not put anything in the mouth; time the seizure; place in recovery position when seizure stops; stay with the youth
Severe allergic reaction / anaphylaxis	Administer EpiPen immediately if prescribed; call 911; follow Section 23 of the Medication Administration Manual
Overdose or suspected poisoning	Call 911 immediately; call Poison Control 1-800-567-8911; do not induce vomiting unless directed; if opioid overdose suspected: administer Naloxone if available and trained; recovery position if unconscious and breathing; rescue breathing if not breathing; notify PC and PM
Severe bleeding	Apply direct pressure with a clean cloth; do not remove the cloth; apply additional cloths on top if soaked through; elevate the limb if possible; call 911 if bleeding is severe or does not slow with pressure; wear gloves
Burn — thermal or chemical	Thermal: cool with cool running water for minimum 20 minutes; do not use ice; remove clothing from burn area unless stuck; call 911 for severe burns (large area, deep, face, hands, genitals). Chemical: brush off dry chemicals first; then flush with large amounts of water for minimum 20 minutes; call Poison Control; call 911
Head injury	Call 911 for any head injury with: loss of consciousness; confusion or disorientation; severe headache; repeated vomiting; vision changes; inability to walk normally. Do not move the youth if spinal injury is



	possible; keep them still until paramedics arrive
Diabetic emergency — hypoglycemia (low blood sugar)	If the youth is conscious and can swallow safely: give 15 grams of fast-acting carbohydrate (3–4 glucose tablets; 175 mL of juice or regular soda; 15 mL of honey). Wait 15 minutes; recheck blood sugar; repeat if still symptomatic. If the youth is unconscious or cannot swallow: do not give anything by mouth; call 911; position in recovery position

22.1 First Aid Requirements

First Aid Requirement	Standard
First Aid / CPR Level C	At least one staff member with current First Aid / CPR Level C certification on duty at all times; certification expiry dates tracked by PC in the Training Compliance Matrix
Occupational First Aid Level 2 (OFA 2)	At least one designated OFA 2 first aider per shift (WCA and OHS Reg. requirement for programs of this size); PM confirms OFA 2 coverage for every scheduled shift
First aid kit	WorkSafeBC-compliant kit; fully stocked and checked monthly (Appendix C); accessible; all staff know its location
AED	Mounted prominently; all staff trained in AED use (included in First Aid / CPR Level C); pads and battery expiry checked monthly

23. MENTAL HEALTH CRISIS AND SUICIDE EMERGENCY

A mental health crisis or suicidal emergency is a medical emergency. The procedures in this section are consistent with and supplement the Behaviour Support Manual (ECC-BSP-2026-001) Section 22.

Situation	Response
Youth expresses suicidal ideation or plan	Stay with the youth; do not leave them alone; remove access to means if safely possible; call the on-call supervisor immediately; contact MCFD social worker; follow the youth's Safety Plan; if the youth has an immediate plan and access to means: call 911
Youth actively attempting suicide	Call 911 immediately; stay with the youth; apply first aid if an injury has occurred; call PC; follow the Direction of paramedics on arrival; notify MCFD immediately; PM notified same day
Youth in acute mental health crisis (psychosis, severe)	Call the on-call supervisor immediately; de-escalate using BSP-documented strategies; call 911 if there is a safety risk to the youth or



dissociation, extreme agitation)	others that cannot be managed; call the mental health crisis line (1-800-784-2433) for guidance; notify PC
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24. OVERDOSE AND POISONING

See also Medication Administration Manual Section 37. This section covers all overdose and poisoning scenarios including non-medication ingestion.

Scenario	Response
Suspected medication overdose — any medication	Call 911 immediately; call BC Poison Control 1-800-567-8911 simultaneously; bring the medication container; do not induce vomiting unless Poison Control directs; stay with the youth; notify PC and PM
Opioid overdose suspected (unresponsive, slow/stopped breathing, blue lips, pinpoint pupils)	Call 911; administer Naloxone (nasal spray: one spray in one nostril; repeat in other nostril after 2–3 minutes if no response); rescue breathing if not breathing; recovery position if breathing; Naloxone wears off in 30–90 minutes — youth must go to hospital
Alcohol intoxication	Call 911 if: loss of consciousness; difficulty breathing; seizure; blue/grey skin. Recovery position if unconscious but breathing; do not leave the youth alone; do not give coffee or food; monitor every 15 minutes; notify PC
Unknown substance ingestion	Call Poison Control immediately; provide as much information as possible about what was ingested and when; follow their instructions exactly; call 911 if directed
Chemical ingestion (cleaning products, medications not prescribed)	Call Poison Control immediately; call 911 if any respiratory symptoms or loss of consciousness; do not induce vomiting for corrosive substances

25. COMMUNICABLE DISEASE OUTBREAK

Outbreak Response Step	Action
Identify the outbreak	Two or more youth or staff with the same illness symptoms within a 48-hour period; notify PC; PC notifies PM; consider whether this meets the threshold for public health notification
Notify Public Health	BC HealthLink at 8-1-1 or the local health authority for guidance; notifiable diseases must be reported by the physician or NP, but the program should coordinate with public health on outbreak management
Infection control measures	Increase hand hygiene frequency (soap and water for at least 20



	seconds; alcohol-based hand sanitizer as backup); increase cleaning frequency for all high-touch surfaces; PPE (gloves, masks) for direct care of ill youth; separate ill youth from well youth where possible
Ill youth management	Ill youth remain in their room where possible; separate bathroom if available; separate eating utensils; staff caring for ill youth take PPE precautions; contact the prescriber if any youth requires medical assessment
Ill staff	Staff who are ill do not attend work; this is non-negotiable during a communicable disease outbreak; see Critical Staffing protocol (Section 31)
Notify MCFD	PC notifies MCFD social workers of any significant outbreak affecting youth in care; PM coordinates with MCFD on any required service changes

26. PANDEMIC AND PROLONGED PUBLIC HEALTH EMERGENCY

Evergreen Community Care's experience with COVID-19 established that pandemic preparedness must be planned for, not improvised. The following framework applies to any declaration of a public health emergency that affects residential care operations.

Pandemic Response Element	Requirement
Follow Provincial Health Officer (PHO) directives	All PHO orders are mandatory; program operations adjust immediately upon any PHO directive affecting residential care; PM receives PHO communications and implements within the required timeframe
Staff screening protocol	Daily health screening of all staff before shift entry; staff with any symptoms of the relevant communicable disease do not attend work; documented
PPE supply	Minimum 2-week supply of required PPE maintained at all times; PC monitors stock; PM authorizes restocking
Vaccination	Programs follow all PHO directives regarding staff and youth vaccination; vaccination status tracked per PHO requirements; religious and medical exemptions managed per HR Policy Binder
Visitor restrictions	Visitor restrictions per PHO directive implemented immediately; youth's right to family contact maintained through alternate means (video call) where in-person visits are restricted; any restriction documented with the PHO directive as authority
Service continuity	PM develops a Pandemic Service Continuity Plan at the onset of any prolonged public health emergency; this plan is submitted to ED and



	MCFD for approval; reviewed and updated at minimum weekly during the emergency period
Documentation	All pandemic-related service changes, PHO compliance measures, and incidents are documented in the program file and reported to MCFD as required by the Service Agreement



PART 7 — Youth-Specific Emergencies

27. MISSING YOUTH — EMERGENCY RESPONSE

A missing youth is an emergency. Every minute matters. The full missing youth protocol is in the RC Program Manual v4 Section 27. This section provides the emergency response framework within the context of this Emergency Preparedness Manual.

Phase	Actions	Timeline
Phase 1 — Prudent Parent Search (maximum 30 minutes)	Immediate facility search (all rooms, yard, common areas); contact known friends and usual locations by phone; check social media for recent activity; notify the shift lead and all staff on site; check that the youth's medications and personal items are accounted for	Begins immediately on discovery; maximum 30 minutes
Phase 2 — MCFD and Police Notification	Call MCFD Provincial Centralized Screening (1-800-663-9122) or the youth's MCFD social worker; simultaneously call the RCMP or local police non-emergency line; provide: youth's name, age, description, last known location, any known associates, risk factors, medications	At 30 minutes or immediately if the youth is known to be at high risk
Phase 3 — PC and PM Notification	PC notifies PM immediately; PM notifies ED if the youth has been missing more than 2 hours or is at high risk; PM coordinates with MCFD on the response	Concurrent with Phase 2
Phase 4 — Ongoing Search and Documentation	Continue to monitor phones, social media, known contacts; document all search steps in a chronological log; cooperate fully with police; do not disclose the youth's care status to media or public	Continuously until the youth is located
Phase 5 — Youth Returns	Welcome the youth warmly; physical health check; call MCFD to confirm return; complete the CI Report; conduct a non-punitive debrief when the youth is regulated; review the BSP	Immediately upon return

28. ABDUCTION AND UNAUTHORIZED REMOVAL

An abduction or unauthorized removal of a youth from care is a criminal matter and an immediate emergency.

Situation	Immediate Response
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Youth is removed by force or by an unauthorized person	Call 911 immediately; call MCFD Provincial Centralized Screening (1-800-663-9122) simultaneously; provide: description of the youth; description of the person; vehicle description (make, colour, licence plate if seen); direction of travel; do not pursue the vehicle
Youth leaves willingly with an unauthorized person	Call 911 and MCFD simultaneously; provide all available descriptions; do not physically block the departure unless you can safely do so; document everything you observed
Youth is threatened or lured by a person outside the facility	Call 911 immediately; lock the facility; keep the youth inside if safely possible; notify PC and PM; do not negotiate with the person outside

⚠ Never disclose the location of a youth in care to any unauthorized person — including family members not on the approved contact list — under any circumstances. Any inquiry about a youth's location that seems designed to facilitate unauthorized access is reported to PC and PM immediately and may be reported to police.

29. YOUTH IN IMMEDIATE DANGER FROM EXTERNAL THREAT

Threat Type	Response
Domestic violence — youth's family member is at the facility and poses a safety threat	Call 911 immediately; implement lockdown if needed (Section 20); do not allow the person inside; keep the youth safe inside; notify PC; notify MCFD; document all details for police
Gang involvement or threat to a specific youth	Notify PC and PM immediately; follow any police or MCFD direction regarding safety planning; review the youth's BSP and safety plan; consider whether enhanced security measures are needed; do not disclose the youth's location to any unknown caller
Online threat or cyberstalking	Notify PC; preserve all evidence (screenshots, messages) before blocking or deleting; notify police; notify MCFD; review the youth's technology access per the BSP
Stranger approaching youth — unknown intent	Move youth inside if they are outdoors; lock the door; call 911 if the person is persistent or threatening; document the incident

30. DEATH OF A YOUTH IN CARE

The death of a youth in care is the most serious event a residential care program can experience. The response must be both operationally correct and deeply human.



Step	Action	Responsible
1. Call 911 immediately	Any unresponsive youth who is not breathing receives CPR (if trained) and an immediate 911 call. Do not assume death has occurred without medical confirmation.	First staff to discover
2. Do not disturb the scene	Once death is confirmed by paramedics: do not move the body; do not clean or disturb anything in the area; paramedics and police will assess and manage the scene	All staff — shift lead directs
3. Support other youth	Other youth in the home need immediate care and support; assign a dedicated staff member to each youth; move other youth away from the scene where possible; honest, age-appropriate information is shared with youth when appropriate — PC advises	Shift lead assigns; PC advises on communication
4. Notify PC and PM immediately	Shift lead calls PC; PC calls PM; PM calls ED — all simultaneously where possible	Shift lead initiates
5. Notify MCFD	PM notifies MCFD Provincial Centralized Screening and the youth's social worker; MCFD will notify the family in most circumstances — do not contact the family before MCFD	PM — within 1 hour
6. Cooperate with police and coroner	A death in care will be investigated; all staff are cooperative and truthful; do not discuss the event with media; redirect all media to ED	All staff; PM leads organizational response
7. Secure all records	All records pertaining to the youth are secured; do not alter, delete, or access unnecessarily; PM provides records to police or coroner as directed by legal counsel and MCFD	PM; legal counsel advises
8. Staff support	All staff involved receive immediate EFAP support; debrief within 48 hours; ongoing support offered; this is a traumatic event for all staff	PM and PC coordinate; ED ensures resources



PART 8 — Staffing Emergencies and Business Continuity

31. CRITICAL STAFFING SHORTAGE

A critical staffing shortage is any situation where the program cannot meet its minimum licensed staffing ratio with currently available staff. This is a safety emergency that must be resolved before it affects the program, not after.

Staffing Shortage Response Step	Action
1. Shift lead is aware of potential shortage	Notify PC as soon as a shortage is anticipated (at minimum 4 hours before shift start if possible; immediately if within-shift emergency)
2. PC activates emergency staffing list	PC contacts all available casual and relief staff in order of availability; all contacts documented (name called, time, response)
3. PM escalation	If PC cannot resolve the shortage within 2 hours: PM is notified immediately; PM contacts additional resources including agency staff where authorized
4. Staff from other programs	PM may temporarily redeploy staff from other programs with appropriate authorization; staff deployed to a program where they are not authorized for specific duties (medication administration) cannot perform those duties
5. On-call supervisor covers in emergency	If no other resolution: the on-call supervisor attends the program in person as a last resort
6. MCFD notification	If minimum staffing ratios cannot be maintained: PM notifies MCFD social worker and PCS (1-800-663-9122) immediately; this is a reportable safety event
7. Documentation	All steps taken to resolve the shortage are documented in the shift log; CI Report completed if minimum ratios were not maintained

⚠ A staff member may NOT leave a shift until a qualified replacement has arrived and received a proper handover. Leaving youth unattended or below minimum staffing ratios constitutes an abandonment of duty and a reportable safety incident, regardless of the circumstances that created the shortage.

32. STAFF INJURY OR INCAPACITATION DURING SHIFT

Situation	Response
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Staff member injured during shift — non-life-threatening	First aid administered; if medical attention is required: call 911 or arrange transport; shift lead assumes all duties for injured staff member; notify PC immediately; arrange coverage before the injured staff member leaves; WorkSafeBC Form 7 if medical treatment required
Staff member incapacitated — medical emergency	Call 911 immediately; apply first aid; notify PC immediately; PC arranges emergency coverage; shift lead manages all youth with remaining staff until coverage arrives; notify PM
Shift lead is the incapacitated person	The most senior available YCW assumes the shift lead role until PC or PM arrives; call PC immediately; PC or PM attends the facility
Staff member appears impaired (alcohol, drugs)	Do not leave the impaired staff member on duty with youth; call PC immediately; PC makes arrangements; this is both a safety emergency and an HR matter; document observations; PM is notified same day; HR Policy Binder s.13 applies

33. PROGRAM MANAGER UNAVAILABILITY

Situation	Response
PM is unavailable during a critical event	PC assumes PM-level decision authority for immediate safety decisions; PC notifies ED; ED provides direction
PM is away for an extended period (leave, illness)	PM designates a named acting PM before any absence; the designation is communicated to all staff and MCFD in writing; the acting PM has full PM decision authority during the absence
Both PM and PC are unavailable simultaneously	ED is notified immediately; ED designates an interim supervisor; this situation should not arise if the on-call schedule is properly maintained

34. BUSINESS CONTINUITY — EXTENDED SERVICE DISRUPTION

A Business Continuity Plan (BCP) activates when normal program operations cannot be maintained at the primary facility for an extended period — due to fire, flood, power failure, building damage, or any other event requiring relocation.

34.1 Pre-Identified Alternate Facility

Each program identifies and confirms an alternate facility for temporary operations before any emergency occurs. This information is documented in the Emergency Documentation Package and confirmed annually.



Alternate Facility Element	Requirement
Location	Alternate facility name and address — confirmed annually with the provider; must be appropriate for residential care (safe, accessible, adequate capacity)
Agreement	A written agreement or letter of understanding with the alternate facility operator is in place; PM confirms currency annually
Transportation	A plan for transporting all youth (including those with mobility needs) to the alternate facility; vehicles and drivers identified
Medications and supplies	Go-bags for each youth are prepared and regularly updated; contain: 72-hour medication supply; essential documentation; comfort items; clothing
MCFD notification	MCFD social workers for all current youth are notified immediately upon activation of the BCP; MCFD approves any change in youth placement or location

34.2 Business Continuity Activation Procedure

Step	Action	Responsible
1. PM declares BCP activation	PM (or ED if PM unavailable) declares that the primary facility cannot be used; BCP is activated	PM — ED informed
2. Notify MCFD	PM notifies all MCFD social workers for current youth and the MCFD Contract Manager; provides alternate location and estimated duration	PM
3. Transport youth to alternate facility	All youth transported safely with their go-bags; Emergency Documentation Package travels with the shift lead	Shift lead coordinates; all available staff assist
4. Establish operations at alternate facility	PC coordinates with alternate facility to set up medication administration, sleeping arrangements, and daily routine; document all temporary changes	PC
5. Communicate with families	MCFD social workers notify families (MCFD leads); program does not notify families independently without MCFD direction in most circumstances	PM confirms with MCFD
6. Plan for return or permanent relocation	PM and ED assess the timeline for return to the primary facility or need for permanent relocation; MCFD kept informed of all developments	PM + ED





PART 9 — Post-Emergency Procedures

35. POST-EMERGENCY DEBRIEF AND RECOVERY

Every emergency event — including false alarms that required an evacuation, near-misses, and incidents that were well-managed — is debriefed before the program returns to normal operations. A debrief is not optional and is not a punishment. It is how the program learns and improves.

Debrief Level	Trigger	Led By
Immediate operational debrief (30 minutes)	Every emergency event, every drill; conducted at the scene or immediately after safe return	Shift lead or PC
Full debrief (within 48 hours)	Any Category 1 emergency; any injury to youth or staff; any event where the response did not go as planned; any event involving police or paramedics	PC or PM
Root cause and improvement review (within 5 business days)	Category 1 events; any event that revealed a gap in the Emergency Preparedness Plan; any event with significant departure from procedure	PM

35.1 Debrief Content

Debrief Component	Questions
What happened?	Factual chronological account; not interpretation; documented in writing
What did we do well?	Genuine recognition of effective actions; specific and named
What did not go as planned?	Without blame; focus on systems and procedures, not individual fault
What does this youth need right now?	Immediate post-emergency care and support for each youth; document in ISP and shift log
What do our staff need right now?	Secondary traumatic stress response; EFAP offered; anyone who needs immediate support identified
What do we change?	At least one specific corrective action with a named owner and target date



36. CRITICAL INCIDENT REPORTING FOLLOWING EMERGENCIES

Many emergency events require a CI Report to MCFD. The following guides the reporting decision. When in doubt: report. Failure to report a required CI is more serious than an unnecessary report.

Emergency Event	CI Category and Reporting Requirement
Fire requiring evacuation	Category 1 — same day verbal to MCFD social worker; CI Report within 24 hours
Any injury to a youth requiring medical attention	Category 1 — same day; CI Report within 24 hours
Natural disaster requiring evacuation or relocation	Category 1 — same day; CI Report within 24 hours
Missing youth	Category 1 — verbal notification at 30 minutes; CI Report on resolution
Abduction or unauthorized removal	Category 1 — simultaneous with 911 call; CI Report within 24 hours
Death of a youth in care	Category 1 — immediate; full CI documentation under MCFD direction
Medical emergency requiring 911	Category 1 — same day; CI Report within 24 hours
Overdose or poisoning	Category 1 — same day; CI Report within 24 hours
Lockdown activation	Category 1 — same day; CI Report within 24 hours
Pandemic or public health emergency affecting operations	Category 2 minimum; PM advises based on PHO requirements
Property damage exceeding \$500	Category 2 — next business day
Power failure exceeding 12 hours affecting youth care	Category 2 — next business day

WorkSafeBC reporting obligations are separate and additional. Any workplace injury requiring medical attention beyond first aid: Form 7 within 3 business days. Any serious incident (fracture, hospitalization, fatality): immediate phone report to WorkSafeBC 1-888-621-7233.

37. TRAUMA SUPPORT FOR YOUTH AND STAFF



37.1 Youth Trauma Support

- Youth who experience an emergency may exhibit: increased anxiety, hypervigilance, changes in behaviour, regression, nightmares, avoidance, aggression, or withdrawal. These are normal trauma responses.
- Staff provide increased relational presence, consistency, and safety following any emergency
- PC contacts the MCFD social worker within 24 hours to discuss any additional clinical support needed
- The BSP is reviewed and updated to reflect any new trauma response patterns observed since the emergency
- Youth are given honest, age-appropriate information about what happened; they are not shielded from all information but are protected from unnecessary detail that could re-traumatize

37.2 Staff Trauma Support

- Responding to an emergency is a significant stressor. Secondary traumatic stress responses are normal and expected.
- EFAP is actively and immediately offered to all staff involved in any emergency — not mentioned once and forgotten
- The post-emergency debrief includes a genuine staff wellbeing check-in; staff are not expected to 'just move on'
- Staff who are experiencing significant secondary traumatic stress are offered modified duties; no staff member is penalized for needing time to recover
- PM monitors staff in the weeks following any significant emergency; supervision sessions address wellbeing explicitly

38. RETURN TO NORMAL OPERATIONS

Return to Operations Step	Requirement
Physical safety confirmed	The facility has been cleared by relevant authorities (fire department, public health, structural engineer, building manager) before youth return
Medications and records reconciled	All medications accounted for; any missed doses managed per prescriber direction; all records recovered or reconstructed
Youth readiness assessed	Each youth's readiness to return to normal routine is assessed; any additional supports in place before return
Staff readiness confirmed	All staff scheduled for the return to normal operations have been debriefed and are ready to return; no staff member returns to duty while in active secondary traumatic stress crisis
MCFD and licensing notified	PM notifies MCFD and the Licensing Officer of the return to normal



	operations; any conditions imposed during the emergency are formally lifted
Corrective actions initiated	All corrective actions identified in the post-emergency debrief are assigned, logged, and in progress
Lessons learned documented	A written summary of the emergency event, the response, and the lessons learned is added to the program's Emergency Preparedness file and informs the next annual review



PART 10 — Training, Drills, and Continuous Improvement

39. EMERGENCY TRAINING REQUIREMENTS

Emergency preparedness training is mandatory for all staff. The following requirements apply before any staff member works an unsupervised shift.

Training	Content	Timing and Renewal
Facility orientation — emergency procedures	Physical walkthrough: all exits; assembly point; fire extinguisher locations; first aid kit; AED; emergency contacts; evacuation plan; Emergency Documentation Package location	Day 1 before first shift; every new program assignment
Fire safety — RACE and PASS	RACE procedure memorized; PASS technique demonstrated; fire extinguisher types; evacuation procedure; role during evacuation	Day 1; reviewed annually with team; practical fire extinguisher training at minimum every 2 years
First aid / CPR Level C	CPR on adult, child, and infant; AED operation; choking response; bleeding control; burns; shock	Before first shift or within 30 days; renewed per certification cycle
OFA Level 2 — designated FAA	Full occupational first aid curriculum; required for at least one staff per shift	Before first duty as designated FAA; renewed per OFA cycle
Earthquake — Drop, Cover, Hold On	Procedure for staff; how to guide youth; post-earthquake assessment	Day 1 orientation; practised in annual earthquake drill
Lockdown / intruder response	Lockdown procedure; door locking; communication with police; keeping youth calm	Day 1 orientation; practised in annual drill
Missing youth protocol	Phase 1 search; Phase 2 notification; documentation; return procedure	Day 1 orientation; reviewed annually; practised in annual tabletop exercise
Naloxone (where applicable)	Signs of opioid overdose; nasal spray administration; recovery position; calling 911	Before first shift where opioids are in use at the program; reviewed annually
EpiPen (where applicable)	Signs of anaphylaxis; EpiPen administration; calling 911; second dose	Before first shift where youth with anaphylaxis risk is in residence; reviewed annually
This Emergency Preparedness Manual — full read and sign	All sections; emphasis on emergency contact directory; evacuation plan; youth-specific emergency information	Before first shift; upon each annual update



Appendix J

40. MONTHLY TRAINING AND DRILL CALENDAR

The Program Coordinator is responsible for ensuring the following emergency preparedness activities are completed each month. The PM reviews and signs off on the training log by the 20th of each month.

Month	Mandatory Activity	Optional / Recommended
January	Monthly safety inspection (Appendix G); 72-hour supply inventory check; emergency contact directory review and update; First Aid / CPR Level C compliance check	Tabletop exercise: major earthquake scenario (discuss response, roles, 72-hour plan)
February	Monthly safety inspection; fire drill (announced) — Q1; smoke and CO detector tests; fire extinguisher visual check; review fire response procedure (RACE) with full team at team meeting	Training: fire extinguisher use practical demonstration for any new staff
March	Monthly safety inspection; 72-hour supply check; NVCI and First Aid expiry review; AED check (pads, battery)	Tabletop exercise: flood or severe weather scenario (shelter-in-place, evacuation decision, alternate facility confirmation)
April	Monthly safety inspection; fire drill (announced) — Q2; emergency lighting test; EpiPen and Naloxone expiry check (where applicable); emergency documentation package review — youth information current?	Training: earthquake response — Drop, Cover, Hold On practice; post-earthquake assessment walkthrough
May	Monthly safety inspection; 72-hour supply inventory; all mandatory training compliance review; hot water temperature check	Tabletop exercise: missing youth scenario (Phase 1 search, MCFD notification, documentation)
June	Monthly safety inspection; fire drill (announced) — Q3; gas shutoff location confirmed with all current staff; emergency contact directory check	Training: lockdown / intruder response drill — walk through procedure with all current staff
July	Monthly safety inspection; annual review of this Emergency Preparedness Manual (Part 10); 72-hour supply restock; MALM and OFA	Annual unannounced fire drill — target night or weekend; evaluate and debrief



	compliance check; update all Appendix H Youth Emergency Information Cards	
August	Monthly safety inspection; fire drill (announced) — Q4; AED expiry check; emergency lighting test; First Aid kit full restock check	Tabletop exercise: critical staffing shortage scenario — who do you call, what if they are unavailable, MCFD notification decision
September	Monthly safety inspection; 72-hour supply check and seasonal review (winter supplies added — blankets, warm clothing, hand warmers); heat and wildfire season debrief	Training: extreme heat response and air quality — review protocol; confirm cooling plan
October	Monthly safety inspection; emergency contact directory full review; fire extinguisher annual professional inspection confirmation; CO detector check as heating season begins	Tabletop exercise: pandemic/communicable disease outbreak scenario — PHO directive response, PPE supply, MCFD notification
November	Monthly safety inspection; 72-hour supply winter check; generator test (where applicable); emergency lighting test	Training: Naloxone refresher (where opioids in use); post-emergency debrief practice
December	Monthly safety inspection; year-end review of all emergency drills conducted — any gaps? any deficiencies unresolved?; confirm all Youth Emergency Information Cards current; prepare annual training compliance report for PM and ED	All-staff emergency preparedness update meeting — review the year, lessons learned, changes for the new year

41. DRILL PLANNING, CONDUCT, AND EVALUATION

41.1 Drill Types

Drill Type	Description	Frequency
Announced evacuation drill (fire)	Staff and youth know a fire drill is coming; full evacuation to assembly point; timed	Quarterly — minimum
Unannounced evacuation drill	No advance notice; simulates a real emergency; conducted at a non-peak time (night, weekend) at least once per year	Annual minimum
Tabletop exercise	Scenario-based discussion drill; staff work	Monthly recommended;



	through the response to a scenario without physically evacuating; excellent for complex scenarios (missing youth, pandemic, earthquake)	scenario rotated through the 12-scenario annual calendar
Functional drill	A specific component of emergency response is practised (e.g., lockdown procedure; Naloxone administration on a training device; EpiPen on a training auto-injector)	As needed; when new procedures are introduced or staff change significantly
Full-scale exercise	A realistic simulation of a major emergency involving all staff and potentially external agencies (fire department may participate); high resource investment	Every 3 years recommended; coordinated with PM and ED

41.2 Drill Evaluation Framework

Evaluation Dimension	Questions to Answer
Time	How long did it take? From alarm to all persons accounted for at assembly point? Is this better or worse than last time?
Completeness	Was every youth and staff member accounted for? Was anyone missed? Was the Emergency Documentation Package retrieved?
Youth response	How did each youth respond? Were any youth particularly distressed or at risk? Did any youth's response reveal a gap in their Emergency Information Card?
Procedure adherence	Did staff follow the procedure? Were any steps skipped? Were any improvised steps taken?
Communication	Was communication between staff clear and effective? Did everyone know what to do? Were there any coordination gaps?
Equipment	Was all emergency equipment accessible and functional? Were there any equipment failures?
Documentation	Was the drill documented immediately after? Does the record capture the key findings?

42. ANNUAL EMERGENCY PREPAREDNESS REVIEW

The Annual Emergency Preparedness Review is conducted each July by the Program Manager. It covers the full emergency preparedness program for the prior 12 months and sets the agenda for the coming year. The review is submitted to the ED and is part of the Annual Program Review (MCFD Standard H.2).



Annual Review Component	Content
Drill and exercise log review	Were all required drills conducted? Were deficiencies identified? Were corrective actions completed? Drill log reviewed for all 12 months.
Emergency incident log review	Were there any actual emergency events? How were they managed? What were the outcomes? What lessons were learned?
Training compliance review	Are all staff current on all emergency preparedness training? Any gaps? Plan to address gaps in the coming year.
Equipment and facility review	All emergency equipment current and functional? Any equipment that needs replacement or upgrade? Facility inspections complete for all 12 months?
Emergency Documentation Package review	Youth Emergency Information Cards current for all current residents? Contact directory current? Alternate facility agreement confirmed?
72-hour supply inventory	Supplies adequate and not expired? Any seasonal gaps identified?
Procedure review	Are any emergency procedures out of date? Any changes in BC Fire Code, OHS Regulation, or MCFD Standards that require procedure updates?
Coming year plan	Updated drill schedule for the coming year; any new training needs; any facility improvements needed; priorities for the emergency preparedness budget
Sign-off	PM signs and dates the review; ED reviews; submitted to Board as part of Annual Program Review; copy retained in the Emergency Preparedness file



PART 11 — Appendices



APPENDIX Appendix A — EMERGENCY CONTACT DIRECTORY TEMPLATE

Print, laminate, and post at every telephone, the main entrance, and the medication storage area. Review quarterly. Update within 24 hours of any change.

EMERGENCY CONTACTS — [Program Name] [Address] Updated:	
911 — fire, police, ambulance	911
BC Poison Control (24/7)	1-800-567-8911
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
BC Crisis Line (24/7)	1-800-784-2433
Representative for Children and Youth (RCY)	1-800-476-3933
On-Call Supervisor — [See posted schedule]	
Program Coordinator — [Name]	[Cell number]
Program Manager — [Name]	[Cell number]
Executive Director — [Name]	[Cell number]
Program Pharmacy — [Name]	[Phone number]
On-Call Physician / NP — after hours	[Number]
BC Hydro — power outage	1-888-769-3766
FortisBC — gas emergency	1-800-663-9911
Local Hospital — [Name]	[Address and number]
Building Owner / Property Manager — [Name]	[Number]
WorkSafeBC — serious incidents	1-888-621-7233



GAS SHUTOFF LOCATION	[Describe location]
ELECTRICAL PANEL LOCATION	[Describe location]
WATER SHUTOFF LOCATION	[Describe location]
ASSEMBLY POINT LOCATION	[Describe location — outside and away from the building]





APPENDIX Appendix B — FACILITY EVACUATION PLAN — TEMPLATE

Complete this template for each floor of the facility. Post on every floor. Update within 30 days of any floor plan change.

Facility Name and Address	
Floor Number	
Date of This Plan	
Prepared By (PC name)	
Approved By (PM name)	

EVACUATION ROUTES	Description
Primary exit from this floor	[Describe: e.g., main staircase at the north end, through the main door, turn left to the rear parking lot]
Secondary exit from this floor (if available)	[Describe]
Assembly point	[Describe exact location: e.g., northeast corner of the rear parking lot, at least 15 metres from the building, marked with a sign]
Fire extinguisher location on this floor	[Describe location]
First aid kit location	[Describe location]
AED location	[Describe location]

ROOM-BY-ROOM EVACUATION NOTES	
Room [Number/Name] — youth with mobility needs	[Youth name or initials; specific assistance required; staff assigned]
Room [Number/Name] — youth who may resist evacuation	[Youth name or initials; de-escalation approach during evacuation; staff assigned]
Room [Number/Name] — notes	



[FLOOR PLAN DIAGRAM — Attach a sketch or printed floor plan showing: all rooms; primary and secondary exit routes marked with arrows; fire extinguisher location marked with symbol; assembly point direction marked; elevator (if any) marked 'DO NOT USE IN EMERGENCY']





APPENDIX Appendix C — EMERGENCY EQUIPMENT CHECKLIST

Complete monthly as part of the facility safety inspection. PC signs and files. PM reviews with the monthly inspection report.

Facility Name	
Inspection Date	
Inspected By (PC name)	
PM Review Date and Signature	

Equipment	Location	Status (OK / Needs Action)	Action Required / Notes
Smoke detector — Floor 1			
Smoke detector — Floor 2 (if applicable)			
Smoke detector — Basement (if applicable)			
CO detector — Floor 1			
CO detector — Floor 2 (if applicable)			
Fire extinguisher — Floor 1			
Fire extinguisher — Floor 2 (if applicable)			
Emergency exit lighting — test activation			
Emergency flashlights (battery check)			
Battery-powered radio (test)			
First aid kit — fully stocked			
AED — pads expiry and battery indicator			
Naloxone kit — expiry check (where applicable)			



EpiPen — expiry check (where applicable)			
Emergency contact directory — current and posted			
Evacuation plan — posted on every floor			
72-hour supply inventory — reviewed (Appendix D)			
Youth Emergency Information Cards — current for all residents			
Emergency Documentation Package — accessible to shift lead			
Hot water temperature — maximum 49 C at all taps			





APPENDIX Appendix D — 72-HOUR EMERGENCY SUPPLY INVENTORY

Reviewed monthly. Quantities sufficient for all current residents plus staff for 72 hours. Restock immediately when items are used or expire. Winter additions marked with (W).

Facility Name	
Number of Youth in Residence	
Number of Staff on Shift	
Review Date	
Reviewed By (PC name)	

Supply Item	Required Quantity	Current Quantity / Expiry
Water — drinking (minimum 4 litres per person per day)	[Total persons x 4 x 3 days = ____ litres]	
Water purification tablets or filter (backup)	1 per kit	
Non-perishable food — 3 days of meals	[3 days x total persons; consider dietary restrictions]	
Manual can opener	1	
Paper plates, cups, cutlery	1 set per person per day x 3 days	
Flashlights with extra batteries	Minimum 1 per floor + 1 backup per person	
Battery-powered or hand-crank radio	1	
First aid kit (backup supplies if primary kit is not portable)	1 per kit specification	
Emergency blankets (mylar / thermal)	1 per person + 2 extra	
Warm blankets (W — winter addition)	1 per person	
Extra warm clothing / tofleece (W)	1 set per person	



Hand warmers (W)	2 per person	
Dust masks / N95 masks	1 per person + 10 extra	
Nitrile gloves	3 pairs per person	
Garbage bags — large (sanitation)	10 large	
Toilet paper	3 days supply per person	
Hand sanitizer (large bottle)	2	
Multi-purpose tool or Swiss army knife	1	
Duct tape and heavy-duty tape	1 roll each	
Cash (small bills — ATMs may be offline)	[\$____] approved by PM	
Youth comfort items — activity books, cards, small toys	Sufficient for 3 days	
Copies of Emergency Documentation Package	Current; stored with supplies	
Medications — 72-hour supply per Individual Medication Profile	Per each youth's current medications	
Phone chargers and backup battery packs	1 per phone in use	





APPENDIX Appendix E — FIRE DRILL RECORD

Complete immediately after every fire drill. Retain in the Emergency Preparedness file for a minimum of 3 years. Submit to PM within 5 business days.

Facility Name	
Date of Drill	
Time of Alarm Activation	
Time All Persons at Assembly Point	
Total Evacuation Time	
Announced or Unannounced?	
Number of Youth Present	
Number of Staff Present	
Were all persons accounted for at the assembly point?	Yes / No — if No, explain:
Was the Emergency Documentation Package retrieved?	Yes / No
Were all exits accessible and unobstructed?	Yes / No — if No, explain:
Did any youth require assistance with evacuation?	Yes / No — if Yes, describe and evaluate:
Were any youth significantly distressed?	Yes / No — if Yes, describe:
Were any procedures not followed correctly?	Yes / No — if Yes, describe:
Overall drill assessment	Satisfactory / Needs Improvement
Corrective actions identified	
Corrective action owner and target date	
Post-drill debrief conducted?	Yes / No Date: _____
PC Signature	



PM Review Signature and Date	
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APPENDIX Appendix F — EMERGENCY DRILL RECORD — ALL TYPES

Complete for every drill or tabletop exercise other than fire drills (which use Appendix E). Retain in the Emergency Preparedness file for minimum 3 years.

Facility Name	
Date of Drill	
Type of Drill	Tabletop / Functional / Announced Evacuation / Unannounced Evacuation / Lockdown / Other:
Emergency Scenario Practised	
Staff Who Participated	
Lead Evaluator (PC or PM name)	
Duration of Drill	
Objectives of This Drill	
Summary of What Was Practised	
What went well?	
What did not go as planned?	
Were any gaps in knowledge or procedure identified?	
Were any youth significantly distressed? (if applicable)	
Corrective actions identified (with owner and target date)	
Overall drill assessment	Satisfactory / Needs Improvement
Post-drill debrief conducted?	Yes / No Date: _____
PC Signature	
PM Review Signature and Date	





APPENDIX Appendix G — MONTHLY SAFETY INSPECTION CHECKLIST — EMERGENCY FOCUS

Complete by the last business day of each month. Submit to PM within 5 business days. PM reviews and signs within 5 additional business days.

Facility Name	
Inspection Month / Year	
PC Name and Signature	
PM Review Signature and Date	

Inspection Item	Standard	Status	Action if Not Met
Smoke detectors — all tested by press-test	Pass every test; document date		Replace battery or unit; retest
CO detectors — all tested	Pass every test; document date		Replace battery or unit; retest
Fire extinguishers — visual inspection	Green zone; pin and seal intact; no damage		Remove from service; arrange inspection
Fire extinguisher — annual professional inspection confirmed	Certificate on file		Schedule inspection immediately
Emergency exit lighting — activation test	All lights activate on power interruption		Replace battery or unit; notify property manager
All exits clear and unobstructed	No furniture, boxes, or other objects blocking any exit		Clear immediately — Priority 1
Exit signs illuminated	All exit signage visible and lit		Replace bulb or battery; notify property manager
Emergency flashlights — battery check	All flashlights functional with fresh batteries		Replace batteries; replace non-functional lights
Battery-powered radio — test	Receives BC Emergency broadcasts		Replace batteries; replace unit
First aid kit — fully stocked per checklist	No items expired or missing		Restock immediately



AED — pads and battery expiry check	Pads not expired; battery indicator OK		Replace pads or battery immediately
Naloxone kit — expiry check (if applicable)	Not expired; accessible outside medication cabinet		Renew at pharmacy immediately
EpiPen — expiry check (if applicable)	Not expired; accessible outside medication cabinet		Replace immediately
Emergency contact directory — current and posted	Posted at all phones and main entrance; all numbers confirmed current		Update within 24 hours of any change
Evacuation plan — posted on every floor	Current version; arrows visible and unobstructed		Replace or update; re-post
Youth Emergency Information Cards — current for all current residents	Updated within 30 days of any change; reflects current ISP		Update within 48 hours
72-hour supplies — sufficient quantity and not expired	Quantities per Appendix D; no expired items		Restock; replace expired items
Hot water temperature — all taps tested with thermometer	Maximum 49 degrees C		Adjust thermostatic mixing valve; notify property manager; Priority 2
WHMIS SDS binder — all chemicals in use have current SDS	All products have SDS; binder accessible		Obtain SDS from supplier within 30 days
Medication cabinet — locked; keys accounted for	Locked at all times except during administration		Investigate; change locks if key unaccounted for





APPENDIX Appendix H — YOUTH EMERGENCY INFORMATION CARD

One card per youth in current residence. Updated within 30 days of any change. Kept in the Emergency Documentation Package accessible to the shift lead at all times.

YOUTH EMERGENCY INFORMATION	
Legal Name	
Preferred Name and Pronouns	
Date of Birth	
Room Number	
MCFD Social Worker — name and phone	
EMERGENCY CONTACTS (authorized persons only)	
Contact 1 — name, relationship, phone	
Contact 2 — name, relationship, phone	
ALLERGY AND MEDICAL ALERTS (prominently listed)	
Known allergies — list all	
EpiPen prescribed? (yes / no)	
Critical medical conditions	
Current prescribed medications — name and time-critical doses	
Prescriber name and after-hours contact	
EMERGENCY-SPECIFIC INFORMATION	
Mobility needs for evacuation	Can evacuate independently / Requires assistance — describe:



Staff assigned to assist this youth during evacuation	
Communication needs (language, AAC device, cognitive level)	
Behavioural response to alarms or emergencies — what to expect	
De-escalation approaches effective during emergency stress	
Behavioural responses to avoid during emergency	
Safety plan triggers or high-risk behaviours during emergency situations	
Items to take in go-bag	Medications; specific comfort item; charger; clothing
DATE OF LAST UPDATE	
PC SIGNATURE	





APPENDIX Appendix I — POST-EMERGENCY DEBRIEF FORM

Complete for every significant emergency event within 5 business days. Retain in the Emergency Preparedness file. Submit to PM; PM submits lessons learned to ED.

Facility Name	
Date of Emergency Event	
Type of Emergency	
Brief description of the event	
Date of This Debrief	
Debrief Facilitator (PC or PM name)	
Staff Who Participated in Debrief	
CHRONOLOGICAL EVENT LOG (what happened, in order)	
Time event began	
Actions taken (in order) — attach shift log if available	
Time resolved or all persons safe	
Total duration of emergency response	
ASSESSMENT	
What did staff do well? (specific and named)	
What did not go as planned?	
Were any procedures not followed? If so, which and why?	
Were there any equipment failures or gaps?	
Were any youth significantly distressed?	



Post-emergency care provided to youth (describe)	
Were any staff injured?	
EFAP offered to all involved staff? (yes / no)	
REPORTING COMPLETED	
CI Report filed with MCFD? (yes / no / not required)	
WorkSafeBC report filed? (yes / no / not required)	
Licensing Officer notified? (yes / no / not required)	
CORRECTIVE ACTIONS	
Corrective action 1 — what; owner; target date	
Corrective action 2 — what; owner; target date	
Corrective action 3 — what; owner; target date	
LESSONS LEARNED — summary for the annual review	
PC Signature and Date	
PM Review Signature and Date	





APPENDIX Appendix J — STAFF EMERGENCY PREPAREDNESS ACKNOWLEDGEMENT FORM

All staff, students, and volunteers must read and sign this acknowledgement before their first shift and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Emergency Preparedness and Business Continuity Manual (ECC-EP-2026-001), Version 1.0, effective July 1, 2026.	
I have completed the facility emergency orientation, including: a physical walkthrough of all exits and the assembly point; location of fire extinguishers, first aid kit, AED, emergency flashlights, and emergency contacts; evacuation plan for my assigned area; role during a fire or other emergency.	
I understand the RACE procedure for fire emergencies (Rescue; Alarm; Contain; Evacuate) and can describe it without reference to this manual.	
I understand the Drop, Cover, Hold On procedure for earthquake response.	
I understand the lockdown procedure and my role if a lockdown is declared.	
I understand that I may not leave my shift until a qualified replacement has arrived, regardless of any personal emergency situation that does not directly affect the facility.	



I understand that in an emergency, the shift lead is the Incident Commander and I follow their direction; if the shift lead is the person incapacitated, I call the PC immediately and assume the most senior role available.	
I know the location of the Emergency Documentation Package and the Youth Emergency Information Cards and can retrieve them without assistance.	
I understand my First Aid / CPR Level C obligation and know my certification expiry date.	
I know the emergency contact numbers for: 911; BC Poison Control (1-800-567-8911); MCFD Provincial Centralized Screening (1-800-663-9122); and my Program Coordinator and Program Manager.	
I will participate in all required emergency drills and training activities.	
Questions about emergency procedures are directed to my Program Coordinator or Program Manager.	

Staff Name (print)	
Position	
Program	
Signature	
Date	
Current First Aid / CPR Level C expiry date	



Current OFA Level 2 expiry date (if designated FAA)	
PC Signature (confirms facility orientation completed)	
Date	

Version History

Version	Date	Summary of Changes
1.0	July 1, 2026	Initial Emergency Preparedness and Business Continuity Manual for Evergreen Community Care; 42 sections in 11 Parts; 10 Appendices; covers fire, natural disaster (earthquake, flood, extreme heat, wildfire), facility emergencies (power failure, gas leak, structural damage, intruder/lockdown, water disruption), medical and clinical emergencies, youth-specific emergencies (missing youth, abduction, death in care), staffing emergencies and business continuity, post-emergency procedures, and a complete monthly training and drill calendar; aligned to WCA, OHS Reg. Part 4, BC Fire Code, RCR Standard F.4, MCFD SHSS Standards F.4/C.1/G.3, CCALA, and Emergency Program Act BC

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____